

Differential Diagnosis

Primary stage lesions of syphilis should be considered in the evaluation of any perianal, anal, or genital ulcer and should be excluded in any ulceration of the lips or oral mucosa.

Secondary Syphilis

Cutaneous Findings

Lesions of secondary syphilis typically erupt 3 to 12 weeks after the appearance of the chancre, although they may develop months later or, in up to 15% of cases, even before the chancre has healed. The secondary stage usually resolves spontaneously within 2 to 12 weeks. Not all patients present with classic symptoms—clinical findings may be subtle and transient, easily overlooked, or in severe cases, require hospitalization. Approximately 60% of patients with latent or late syphilis report no prior signs or symptoms of secondary disease, and one-fourth cannot recall the presence of a primary chancre.

Patients may experience systemic symptoms such as malaise, fever, headache, stiff neck, myalgias, arthralgias, anorexia, lacrimation, nasal discharge, and depression. However, between 80% and 95% of cases present with a skin eruption, which is the hallmark diagnostic feature.

Organ involvement varies across studies. In a series of 2,269 cases of secondary syphilis, involvement was reported as follows: - Skin: 81.1% - Oral cavity and pharynx: 36.3% - Genitalia: 19.9% - Central nervous system (CNS): 9.9% - Eyes: 4% - Visceral organs: 0.2%

Given its varied presentation, syphilis is often referred to as the "great imitator," and should be considered in any unexplained constellation of signs and symptoms.

Most skin eruptions in secondary syphilis are macular and/or papular. Nodular and pustular forms occur infrequently, while vesiculobullous lesions are exclusive to congenital syphilis. Early lesions are typically symmetric, becoming

polymorphic over time. Pruritus may be more common in individuals of African descent. Non-ulcerated lesions heal without scarring, with or without treatment, within 2 to 12 weeks.

Macular Eruptions (Roseola Syphilitica)

Macular eruptions consist of 0.5- to 2.0-cm pink, discrete, non-scaling, oval macules and patches. They predominantly affect the trunk and flexor surfaces of the upper extremities. The face is usually spared, but lesions can appear on any skin surface, including the palms and soles.

Macular and Papular Eruptions

These represent the evolution of macular lesions into palpable papules and plaques. Over time, some macules become elevated and develop a characteristic dark coppery hue. Lesions commonly appear on the genitalia, face, palms, and soles. Those clustered around the hairline may form a crown-like pattern known as *corona veneris*.

Papular Eruptions

Papular eruptions include several subtypes: - Papulosquamous - Follicular - Lenticular - Corymbose ("bombshell") - Nodular - Annular - Follicular

They may be generalized (see Fig. 200-9) or grouped, aggregated, and localized to specific areas (Figs. 200-10 and 200-11).

Papulosquamous eruptions feature discoid, copper-colored or erythematous, oval or circular, indurated papules or plaques with a flat, shiny, scaly surface (see Fig. 200-10).

Box 200-1: Differential Diagnosis of Primary Syphilis

Most Likely

- Chancroid
- Herpes simplex

- Granuloma inguinale
- Traumatic erosion or ulcer

Consider

- Herpes simplex
- Early lymphogranuloma venereum
- Chancriform pyoderma
- Aphthous ulcer
- Behçet disease
- Squamous cell carcinoma
- Basal cell carcinoma
- Fixed-drug eruption
- Erosive candidal vulvitis or balanitis